

THE SEND NAVIGATE LIBRARY



The School Distress Operating Workbook

*The free companion to
When Your Child Can't Go to School*

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How to use this workbook

This is a working document, not a journal. Each tool inside it is designed to be opened alongside the book – and to be used in the morning when something is unmanageable.

The order is deliberate

The tools are arranged in the same order the book is. Stabilise the child first (the Three-State Tracker), then the home (the Core Demands Audit), then the language you use with the school (the Language Audit), then the legal evidence (the Symptom and Impact Log), then the formal letters (Section 19, EHCNA, Exclusion).

You do not need to use them in this order. Use the one that matches today.

A note on the Situation Map

If you complete only one page in this workbook, complete the Situation Map at the end. It is the one page that summarises your case – bring it to the free 15-minute call and it will be twice as useful.

A NOTE ON CRISIS

If your child is in immediate danger – call **999**. If your child is in acute distress but not immediate danger – call **NHS 111 and select Option 2** for the mental-health crisis line. The rest of this workbook is for parents with time to work through it. If you do not have time, that is not what you need today.

How to fill it in

Print it, write on it, photograph the pages you complete. Or open it in a PDF reader (Adobe Acrobat, Preview, Foxit) and type into the fillable areas. Both work.

What this workbook is not

It is not legal advice. It is a structured set of working tools that go alongside professional support. Where a tool generates a letter, that letter is a starting point – the language must be adapted to your child's specific situation before sending.



SECTION 1

Understanding your child's state

*Read what their body is saying. Apply the rule for
that state.*

The single most important practice in this workbook.

The Three-State Tracker

Every child in school distress lives in one of three nervous-system states at any given moment. Reading the state is something you learn within a fortnight. Applying the rule for that state is what changes everything.

GREEN – VENTRAL VAGAL • SAFE AND SOCIAL

What it looks like

- **Body:** fluid movement, normal posture, open shoulders, regulated breathing, soft eye contact
- **Voice:** normal volume, fluent speech, willing to answer questions, occasional laughter
- **Behaviour:** initiates contact, plays, eats, sleeps in a familiar pattern, asks for things
- **Energy:** present but not hyper, engaged with what's around them

If you see Green, do this

- **Use the window.** Green is the only state in which to introduce conversation about school, the future, or expectations. Even then, lightly.
- **Reinforce safety.** Sit with them. Watch a screen together. Cook together. Build their sense that home is where they are accepted as they are.
- **If you raise something difficult, name it as a question.** "I was wondering if it would feel OK to..." not "We need to talk about..."
- **Do not flood it.** Green is precious. Doing too much in a Green window pushes them back to Amber by evening.

If you see Green, do NOT do this

- Pile up agendas — even good ones
- Test whether they could "just try" something they couldn't yesterday
- Talk about plans more than ten minutes at a stretch

What it looks like

- **Body:** pacing, fidgeting, clenched jaw, raised shoulders, fast breathing, defensive posture
- **Voice:** raised, sharp, snapping, talking over you, repetitive demands, sudden silence
- **Behaviour:** door slamming, throwing things, refusing food or water, hyper-fixation on screens, refusal to remove a coat or shoes
- **Energy:** spiking, unstable, prone to sudden escalation

If you see Amber, do this

- **De-escalate immediately.** Lower your voice. Slow your movement. Drop the demand that triggered it.
- **Offer parallel presence.** Sit nearby without facing them directly. Side-by-side, not face-to-face.
- **Offer one regulating action only.** A drink of water. A blanket. A short walk. Phone. Not a list.
- **Wait it out.** Sympathetic states usually pass within 30 to 90 minutes if no new demand is added.

If you see Amber, do NOT do this

- Reason with them, explain, or list consequences
- Talk about school, the future, or expectations
- Match their volume or speed
- Touch them without permission
- Send them to their room as a punishment (different from them choosing to retreat)

What it looks like

- **Body:** immobility, slumped posture, low muscle tone, slowed breathing, eyes unfocused or shut
- **Voice:** selective mutism, monosyllables, no response to questions
- **Behaviour:** can't get out of bed, can't dress, can't eat, can't shower, sleeping during the day, withdrawal from previously regulated activities
- **Energy:** flat, collapsed, depleted

If you see Red, do this

- **Zero demand.** Treat this like the day after a severe concussion. No questions, no requests, no expectations.
- **Make basics accessible without interaction.** Leave water, snacks, soft food, a charger, a blanket within reach.
- **Sit in the same room.** Do not require eye contact, conversation, or acknowledgement. Presence is enough.
- **Maintain sensory rest.** Lower lights, lower volume, remove notifications if you can.
- **Log it.** Use the Symptom & Impact Log to record the date, duration, and observable signs. This is evidence.

If you see Red, do NOT do this

- Ask "How are you feeling?" — they cannot access the answer
- Plan for tomorrow with them present
- Allow visitors or unannounced calls
- Mention school, even gently
- Try to feed them on a schedule

A child can move through all three states in a single day. Reading their state at the moment of the demand is more important than reading their state at breakfast.

This Week's Tracker

One row per day. In each cell, write the dominant state – G, A or R. Notice patterns: when in the day do they shift? What precedes it? You will see a pattern within a fortnight.

DAY	MORNING	MIDDAY	AFTERNOON	EVENING	TRIGGER OR NOTE
Mon					
Tue					
Wed					
Thu					
Fri					
Sat					
Sun					

PATTERNS I NOTICED THIS WEEK

WHAT I WILL CHANGE NEXT WEEK



SECTION 2

Stabilising the home

*Reduce the demand. Remove the triggers.
Make safety unmistakable.*

The Core Demands Audit

Every demand placed on a child in distress is a small additional pressure on a nervous system that is already at the edge. This audit asks you to look at every demand in your home and reduce as many as possible for a set period.

DEMAND AREA	SCORE 1 – 5	WHAT I WILL DROP OR REDUCE THIS WEEK
Morning routine (waking, dressing, breakfast)		
Sleep / bedtime		
Hygiene (shower, teeth, dressing)		
School conversations		
Schoolwork at home		
Mealtimes (where, when, what)		
Sibling interactions		
Screen time rules		
Visitors and family events		
Tidiness or chores		
Direct questions ("How are you feeling?")		
Other		

THE PRINCIPLE

If a demand does not relate directly to immediate physical safety, it can be paused for a period. This is not parenting failure. It is a temporary clinical intervention.

You are not abandoning structure. You are reducing the cortisol load while the nervous system recovers. Once they are spending more days in Green than Amber, demands can re-enter slowly.

The three top-pressure demands I will drop this week

1.

2.

3.

What I expect to change in two weeks

The Trigger Sweep

Many of the most damaging triggers are visible objects the child sees every day. Pack them away, out of sight, for the duration. This is not denial. It is removing daily re-traumatisation.

Things to put out of sight today

- ☐ School uniform – drawer or wardrobe, closed
- ☐ School bag, book bag, lunch bag
- ☐ Reading record, homework planner, exercise books
- ☐ School shoes
- ☐ Class photograph or any class group communication
- ☐ Letters from school (open elsewhere; do not leave on the table)
- ☐ Notification sounds from school apps (ParentMail, Class Dojo, etc.) – mute on your phone
- ☐ Calendar reminders about school events

Things to put in their place

- ☐ A basket of neutral materials they choose (Lego, drawing, soft toys, books they like)
- ☐ A blanket on their bed or chair that signals "this is yours"
- ☐ Snacks and water within reach without asking

WHY THIS WORKS

For a child in chronic school distress, the school uniform on the hook is not a clothing item. It is a daily exposure to the source of harm. Removing it is the same principle as removing a phobic stimulus from a person undergoing recovery – you do not desensitise by repeated exposure to the thing that triggers the response.



SECTION 3

The language that protects you

*Every email you write from emotion gives the system
a stick to beat you with.
Rewrite the language. Shift the burden.*

The Language Audit

The single most consequential thing in your written communication with the school and the Local Authority is the language you use. Below: the most common emotional phrasings, the legal-armour translation, and why the swap matters.

INSTEAD OF (EMOTIONAL)	WRITE (LEGAL ARMOUR)	WHY
"He is refusing to come in."	"He is currently medically unfit to attend."	"Refusal" implies choice. "Medically unfit" triggers Code I (Illness).
"I can't get him out of bed."	"His severe anxiety has resulted in neurological immobilisation."	Confirms physical impossibility, not parental discipline failure.
"She is too anxious to come in."	"She is experiencing acute EBSA driven by an unsuitable environment."	"Anxiety" can be dismissed. EBSA is a recognised clinical profile.
"I'm so sorry for the short notice..."	"Please find below the formal medical update regarding..."	Never apologise. You are managing a medical emergency, not asking a favour.
"He just won't do what he is told."	"He is currently in a Dorsal Vagal freeze state. Behavioural consequences are medically contraindicated."	Naming the clinical state shifts the framing from discipline to neurology.
"I don't know what to do anymore."		Names a strategy.

INSTEAD OF (EMOTIONAL)	WRITE (LEGAL ARMOUR)	WHY
	"We are currently operating a clinical low-demand framework at home, in line with [GP/practitioner] advice."	Removes the impression you are without a plan.
"He hates school."	"The school environment is currently inducing acute distress that exceeds his capacity to access learning."	"Hates" is feelings. The new phrasing is a functional and clinical statement.
"Please help us, we're desperate."	"I am writing to formally request a multi-agency meeting under Section 19 of the Education Act 1996."	Statutory request, not emotional plea. The system responds to statutory triggers.
"We tried but it didn't work."	"The proposed intervention triggered immediate neurological regression. Continued attempts at this approach are clinically contraindicated."	Documents the FAILURE of their plan, not the failure of your effort.
"He's been off for ages now."	"He has been unable to access school for [X] cumulative days, exceeding the statutory threshold under DfE guidance ('Ensuring a good education...')."	Triggers Section 19 duty. The 15-day rule is your friend.

THE GOLDEN RULE

Before sending any email, read it aloud and ask: would this sound like an irresponsible parent in a courtroom, or like an authoritative professional managing a healthcare crisis? If the latter, send.

Now translate yours

Write three emotional phrases you've found yourself using. Translate each into legal armour. The translations get easier with practice.

INSTEAD OF

WRITE



S E C T I O N 4

Building the evidence

*Subjective parental worry will be dismissed.
Clinical-grade documentation cannot be ignored.*

The Symptom & Impact Log

One entry per day. Cold clinical facts only – no narrative, no emotion. Three to four weeks of entries make this log inarguable. Bring it to every GP appointment and attach it to every formal letter.

EXAMPLE – A COMPLETED ENTRY

Date: Mon 12 May 2026

Physiological: Severe nausea 07:00–08:30. Dry-heaving over basin. Resting heart rate 115 bpm. Unable to eat breakfast. No fluids until 11.00.

Psychological / behavioural: Selective mutism from 06:45. Locked bedroom door 07:30. Hyperventilation when uniform laid out. Eventually slept until 13.00.

School touchpoint / trigger: Sunday evening automated school attendance text. Anticipated Monday assembly.

Action taken: Sent medical absence email under Code I at 08.45. Cancelled all demands. Sat in shared space until child re-emerged.

Today's entry

DATE

PHYSIOLOGICAL (SLEEP, EATING, DRINKING, VOMITING, HEART RATE, PANIC, SOMATIC SIGNS)

PSYCHOLOGICAL / BEHAVIOURAL (MUTISM, WITHDRAWAL, MELTDOWN, REGRESSION, SELF-HARM FLAGS)

SCHOOL TOUCHPOINT OR TRIGGER

ACTION TAKEN

Continued — another day

Photocopy this page for the next 28 days, or use the digital fillable version. The pattern matters more than any single entry.

DATE

PHYSIOLOGICAL

PSYCHOLOGICAL / BEHAVIOURAL

SCHOOL TOUCHPOINT OR TRIGGER

ACTION TAKEN

DATE

PHYSIOLOGICAL

PSYCHOLOGICAL / BEHAVIOURAL

SCHOOL TOUCHPOINT OR TRIGGER

ACTION TAKEN



SECTION 5

The legal pathways

*Three routes the system rarely tells you about.
Each one is a statutory duty, not a request.*

The Section 19 Formal Demand Letter

Section 19 of the Education Act 1996 places an absolute statutory duty on the Local Authority to provide suitable, full-time education for any child who, by reason of illness or otherwise, cannot otherwise receive a suitable education. Send this letter directly to the Director of Children's Services. Bypass the school.

BEFORE YOU SEND, CONFIRM

- ☐ My child has been out of school for 15+ days cumulative this year
- ☐ I have a GP fit note (Med 3) on file, or one is being arranged
- ☐ I have at least 14 days of Symptom and Impact Log entries
- ☐ I have evidence the school's strategies (PE, friend-led, part-time) have been attempted or refused
- ☐ I am ready to email this directly to the Director of Children's Services at my LA

FILLABLE TEMPLATE

Subject: FORMAL DEMAND: Provision of Alternative Education under Section 19 Education Act 1996 – [Child's Name], [DOB]

To the Director of Children's Services,

I am writing to formally notify you that the Local Authority is currently in breach of its statutory duty under Section 19 of the Education Act 1996 in respect of my child, [Child's Name], [DOB].

[Child's Name] is currently out of school and has been unable to access his/her named placement, [School Name], for [Number] cumulative weeks due to medical unfitness driven by severe mental health distress and Emotionally Based School Avoidance (EBSA).

The Local Authority has been aware – or ought to have been aware – that [Child's Name]'s absences have significantly exceeded the statutory threshold of 15 days outlined in DfE guidance ("Ensuring a good education for children who cannot attend school because of health needs").

I enclose an Evidence Binder, including a formal Med 3 fit note from our GP confirming that [Child's Name] is currently medically unfit to attend a school building. In line with case law (*G v Westminster City Council* [2004]), the Local Authority cannot ignore this medical evidence, nor condition the provision of

alternative education on a multi-year waiting list for a tertiary psychiatric diagnosis.

I hereby demand that the Local Authority:

1. Formally acknowledge its Section 19 duty regarding [Child's Name] within 5 working days.
2. Immediately commission an interim alternative education package (such as an accredited online learning platform or 1:1 home tuition) to halt his/her educational deficit.
3. Cease and desist any punitive attendance enforcement or fine procedures, as the absences are fully documented as due to medical sickness under Section 444(2A).

If a compliant, costed alternative provision framework is not proposed within 10 working days of this letter, I will escalate this matter to the Local Government and Social Care Ombudsman (LGSCO) and seek legal counsel regarding a Judicial Review.

Kind regards,
[Your Name]
[Your Address]
[Date]

IF YOU WOULD RATHER NOT SEND THIS ALONE

The Initial Consultation (£160) at SEND Navigate includes drafting guidance and a written legal-position summary for your specific situation. The free 15-minute call is unconditional.

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The EHCNA “May” Test Checklist

Under Section 36(8) of the Children and Families Act 2014, the Local Authority must conduct an EHC Needs Assessment if BOTH tests below are met. The legal threshold is deliberately low – the word is "may", not "definitely".

Test 1 – Does the child have or MAY have SEN?

Tick all that apply. Any one is enough.

- ☐ Documented anxiety, EBSA, or school distress
- ☐ Autism diagnosis or suspected autism
- ☐ PDA profile
- ☐ ADHD diagnosis or suspected ADHD
- ☐ Trauma history (medical, social, or environmental)
- ☐ Sensory processing difficulties
- ☐ Speech, language or communication difficulties
- ☐ Specific learning difficulties (dyslexia, dyscalculia, dyspraxia)
- ☐ Long-term physical health condition affecting access to school
- ☐ Currently out of school 15+ days due to health needs

Test 2 – MAY it be necessary for special educational provision to be made?

Tick all that apply. Any one is enough.

- ☐ School's existing strategies have not worked
- ☐ Standard reasonable adjustments are insufficient
- ☐ The current environment causes a physiological reaction
- ☐ Specialist provision (online, alternative, or therapeutic) may be required
- ☐ The level of support needed exceeds what is normally available in a mainstream classroom

YOUR RESULT

If you ticked at least one box in Test 1 AND at least one box in Test 2, the legal threshold for assessment is met. The Local Authority must assess. The application is a direct parental right under Section 36 – you do NOT need the school's permission.

Over 90% of refusal-to-assess appeals are won by parents. Almost all are conceded by the LA before a hearing is held.

The Exclusion & Discrimination Quick Check

If your child has been excluded for behaviour related to a disability, or if the school has refused reasonable adjustments, you may have a separate legal route under the Equality Act 2010 – distinct from the Section 19 / EHCP route.

Has any of this happened?

- ☐ Your child has been formally excluded (fixed-term or permanent)
- ☐ Your child has been informally "sent home" or asked to be collected repeatedly
- ☐ A part-time timetable has been imposed without your written consent
- ☐ Your child has been internally isolated (Reflection Room, Inclusion Unit, etc.) regularly
- ☐ Reasonable adjustments you have requested have been refused or ignored
- ☐ Your child has been disciplined for behaviour that is a manifestation of their disability
- ☐ You have been threatened with managed moves or off-rolling
- ☐ Your child has been treated unfavourably because of their disability or neurodiversity

IF YOU TICKED ANY OF THE ABOVE

You may have grounds for a disability discrimination claim under the Equality Act 2010. The statutory time-limits are tight: **6 months from the date of the incident** for a SEND37 tribunal claim; **15 school days** from a fixed-term exclusion notice to request a governors' independent review.

This route is separate from a Section 19 / EHCP appeal. Both can run in parallel.

IF YOU WOULD RATHER NOT NAVIGATE THIS ALONE

SEND Navigate offers an Exclusion & Discrimination Support package (£1,200) covering full case review, drafting of the SEND37 claim OR the governors' independent review submission, Equality Act-grounded correspondence, statutory deadline tracking, and coaching through the process. The free 15-minute call will tell you whether it applies.

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IF THIS MIGHT APPLY TO MY CASE, THE DATES I NEED TO TRACK ARE



SECTION 6

Your Situation Map

*A single page summarising your case.
The one page you bring to a free 15-minute call.*

Your Situation Map

The page that summarises everything. Fill in what applies. Bring this to any conversation – paid or free – and you'll save half an hour of discovery and get to the next move twice as fast.

DATE

YOUR CHILD

AGE

YEAR GROUP

DAYS OUT OF SCHOOL

CURRENT DOMINANT STATE (G / A / R)

DIAGNOSES OR SUSPECTED PROFILES

THE SCHOOL

SCHOOL NAME

MAINSTREAM / SPECIAL / INDEPENDENT

SCHOOL'S CURRENT POSITION

LAST FORMAL COMMUNICATION RECEIVED

THE LOCAL AUTHORITY

YOUR LA

LA'S CURRENT POSITION

WHAT YOU HAVE

- ☐ Med 3 fit note from GP
- ☐ 3+ weeks of Symptom & Impact Log
- ☐ Evidence Binder assembled
- ☐ Section 19 letter drafted
- ☐ Section 19 letter sent
- ☐ EHCNA application submitted

ANY EHCP, DRAFT, OR REFUSAL-TO-ASSESS ON FILE

- ☐ EHCNA decision received
- ☐ Existing EHCP
- ☐ Draft / proposed EHCP under review

WHAT YOU WANT NEXT — in one sentence

THE SINGLE THING BLOCKING YOU RIGHT NOW

Your Next Step

You have a choice. Both are valid. Most parents move between the two.

Option A — Continue alone.

Use the book and this workbook. Come back to your Situation Map in a fortnight and update it. The book has the rest of what you need.

Option B — Take the free 15-minute call.

Bring your Situation Map. We'll talk through where you are and what would help next. No pressure. No sales pitch. Just an honest conversation.

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Resources beyond SEND Navigate

UK-wide

- **SENDIASS** — free Local Authority-funded independent SEND advice for parents
- **IPSEA** — free legal advice on EHCPs and tribunals (ipsea.org.uk)
- **Square Peg** — UK school-attendance campaigners (teamsquarepeg.org)
- **Not Fine In School** — UK parent community (notfineinschool.co.uk)
- **PDA Society** — Pathological Demand Avoidance support (pdasociety.org.uk)

Scotland

- **Enquire** — funded parental advice service (enquire.org.uk)

Wales

- **SNAP Cymru** — independent parent partnership (snapcymru.org)

Northern Ireland

- **Parenting NI** — family advice and support (parentingni.org)

*You did not fail your child.
You are at the start of a different kind of fight,
and there are routes through it.*

— Stephen Mallett
Founder, SEND Navigate
Author of *When Your Child Can't Go to School* and *Parent Shield*